INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab intravenous (IV) · every 3 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00200
Version 8

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

Treatment stages

01 **Early-breast pathway**
One year total

02 **Alternative: metastatic pathway**
Ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Trastuzumab intravenous (IV) · every 3 weeks · National Cancer Control Programme (NCCP) 00200

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report breathlessness, swelling, rapid weight gain, palpitations or persistent diarrhoea; tell staff immediately about a reaction during treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

The medicine in this card

In early breast cancer, trastuzumab helps lower the chance of recurrence. In metastatic disease, the aim is ongoing cancer control. Your team should name which pathway applies before discussing duration.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Attend planned echoes. Report breathlessness, ankle swelling, rapid weight gain or infusion symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

Your cancer has a feature called HER2 that this medicine targets. In early breast cancer, treatment aims to lower the chance of the cancer returning. In metastatic disease, it aims to help control the cancer. These are different pathways, with different treatment durations.

Why this schedule?

This is the three-weekly intravenous schedule. Your team considers your treatment setting, earlier doses, other medicines, heart health and visit arrangements. It is not a claim that three-weekly treatment works better than weekly treatment.

Your own decision

Ask which cancer features, earlier treatments and health factors led to this recommendation, what alternatives were considered, and when the plan will be reviewed.

Your treatment aim

Setting and duration

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.



Medicine identity

This card describes trastuzumab. Pertuzumab, taxanes, endocrine medicines and antibody–drug conjugates require their own treatment instructions.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Trastuzumab IV · every 3 weeks · NCCP 00200

Where this treatment fits

Treatment stages

01 **Early-breast pathway**
One year total

02 **Alternative: metastatic pathway**
Ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE YEAR TOTAL

Early-breast pathway

Early disease

Count all preceding trastuzumab doses; complete the agreed year unless recurrence or toxicity changes the plan.

ONGOING REVIEW

Alternative: metastatic pathway

Metastatic disease

Continue while beneficial and tolerated. This is an alternative indication, not a second phase after completing early-breast therapy.

Early and metastatic pathways have different aims and endpoints. The team confirms the applicable plan.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. NCCP remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment. After stopping: six-monthly to 24 months; after anthracyclines in early disease, yearly to 5 years or longer if needed.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Additional tests if needed	Team / companion-drug plan	Team / companion-drug plan	Repeat according to the team / companion-drug plan; no fixed universal interval in this NCCP card.
electrocardiogram (ECG) / rhythm	Baseline ECG in EMA cardiac assessment; it does not replace echo.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Document the disease sites and imaging purpose	Assess symptoms at visits	Scan only if due / indicated	Metastatic: often near a response review; exact date agreed	Early: breast surveillance, symptom-led additional tests. Metastatic: agreed response scans, commonly every 2–4 months; markers are supplementary.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	If endocrine / ovarian suppression is planned: assess need	Not a routine trastuzumab or T-DM1 test alone	Timing depends on risk / result	Separate from the echo schedule	DEXA and repeat timing depend on endocrine treatment, fracture risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00200 v8 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · cardiac assessment and formulation

THE WHOLE TREATMENT JOURNEY

Trastuzumab IV · every 3 weeks: the whole pathway

HER2-positive early or metastatic breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00200	Confirm setting, prior doses and formulation	IV trastuzumab every 3 weeks	Review benefit, symptoms and heart function	Early: complete one year total. Metastatic: ongoing review.	Early: completed year if applicable. Metastatic: review whether to continue or change.
Surgery EARLY	Early: surgery may precede or follow initial systemic therapy	Neoadjuvant route: reassess before planned surgery	multidisciplinary team (MDT) reviews recovery and pathology	Early: continue prescribed postoperative HER2 plan	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Early: usually after chemotherapy ; coordinate surgery	May overlap trastuzumab continuation if agreed	Early: radiotherapy (RT) may overlap trastuzumab continuation, when agreed.	Radiation effects and surveillance follow the local plan

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Blood tests NCCP 00200	Baseline FBC, renal, liver	Repeat according to the team / companion regimen	According to the team / companion regimen	Repeat according to the team / companion regimen	Follow abnormalities and the next treatment plan
Heart checks NCCP 00200 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail.
Scans / markers EARLY · METASTATIC · MARKERS	Define baseline sites and scan purpose	Review symptoms at each visit	Metastatic response imaging if due; not an automatic early-breast computed tomography (CT)	Individualised response or breast surveillance plan	Early: breast surveillance, symptom-led additional tests. Metastatic: agreed response scans, commonly every 2–4 months; markers are supplementary.
Endocrine / bone health EARLY · BONE	Check hormone receptor status and endocrine indication	Endocrine timing coordinated with the wider plan	DEXA if indicated by endocrine / fracture risk	May overlap trastuzumab when clinically appropriate	Longer endocrine and bone follow-up can outlast HER2 treatment

What each check is for

1

Blood tests

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here)

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

2

Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

3

Cancer imaging and markers

Early disease: planned breast surveillance; no routine CT or tumour-marker screening for recurrence without symptoms. Metastatic disease: response imaging on an agreed schedule, commonly every 2–4 months; earlier for new concerns.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used according to the treatment plan. A port insertion is a separate procedure.

STEP 3

Treatment and observation

First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit's approved policy.

STEP 4

After the visit

Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose. This medicine does not routinely require a take-home pump.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Before treatment and between visits

The team should arrange and review the tests.

Blood checks

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here)

Cardiac checks

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Baseline cardiac assessment includes ECG in EMA trastuzumab information. Further ECGs follow symptoms / cardiac plan; an ECG does not replace an echo.

After stopping

EMA trastuzumab information: cardiac assessment every 6 months after stopping, until 24 months; after anthracyclines in early disease, yearly to 5 years or longer if function falls.

The wider pathway

Coordinate cancer medicines with local treatment and follow-up.

Scans and markers

Early disease: planned breast surveillance; no routine CT or tumour-marker screening for recurrence without symptoms. Metastatic disease: response imaging on an agreed schedule, commonly every 2–4 months; earlier for new concerns.

Radiotherapy relationship

If postoperative radiotherapy is indicated, it usually follows chemotherapy and may overlap with trastuzumab continuation under the MDT plan. The radiation team assesses heart / lung exposure. Endocrine therapy may also overlap when indicated. In metastatic disease, radiotherapy is site / symptom directed and needs its own coordination plan.

Bone health

If an aromatase inhibitor or ovarian suppression is planned, assess fracture risk and DEXA need. Repeat timing depends on results and bone treatment; this is not an automatic HER2-therapy scan.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Trastuzumab IV · every 3 weeks · NCCP 00200

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment	 **Day 1** Planned dose
Administration reaction	 **Day 1** During / after visit; report promptly
Fatigue / bowel changes	 **Days 1–21** Timing and severity vary
Heart / breathing symptoms	 **Days 1–21** Report on any day

No date entry needed: This is an orientation map; it does not calculate a personal appointment calendar.

Companion medicines: If another cancer medicine is prescribed, its side effects, tests and supportive medicines need its own plan.

Delayed treatment: Blood tests and echoes follow clinical due windows. A postponed dose does not automatically postpone monitoring.

Why these bands stay broad: Treatment days show the planned schedule. Symptom bands are broad possibilities, not measured risk curves or guaranteed safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00200 v8 · official regimen ↗

There are no guaranteed "safe days". New breathlessness, chest pain, collapse, severe allergic symptoms or sudden serious illness need urgent assessment on any day.

Between-treatment notes

Day 1–21: Your reference cycle

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop. The team confirms each dose after the relevant checks. Contact them about a missed visit or new symptoms.

Support around the medicines

1 

Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

2 

Blood and heart appointments

Baseline FBC, renal and liver profile; repeat blood tests according to the team / companion-regimen plan (NCCP does not assign a universal interval here). Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles.

3

Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the MDT. When endocrine therapy is indicated, it has its own schedule and bone-health plan.

4

Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Questions worth asking

When is my next echo?

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Existing HER2 exposure and delays affect cycle numbering; the team should give the actual due window.

Does a tumour marker replace a scan?

No. In metastatic disease it may add information to symptoms and imaging. It should not alone determine treatment changes. In symptom-free early disease it is not routine recurrence screening.

Why was this treatment chosen for me?

Your cancer has a feature called HER2 that this medicine targets. In early breast cancer, treatment aims to lower the chance of the cancer returning. In metastatic disease, it aims to help control the cancer. These are different pathways, with different treatment durations. This is the three-weekly intravenous schedule. Your team considers your treatment setting, earlier doses, other medicines, heart health and visit arrangements. It is not a claim that three-weekly treatment works better than weekly treatment.

> **Try explaining the plan back:** What is the purpose of your treatment? Which heart or blood test is due next, and which team arranges and reviews it?

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Trastuzumab IV · every 3 weeks

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.
- First infusion usually 90 minutes; later infusions may take 30 minutes if tolerated. NCCP observation is 6 hours from the start of the first infusion and 2 hours from the start of later infusions; ask about the unit's approved policy.
- Tell the unit how late the dose is. A delay over one week can require re-loading after clinician review; do not assume every continuation visit needs a loading dose.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00200 · VERSION 8

Trastuzumab IV · every 3 weeks

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

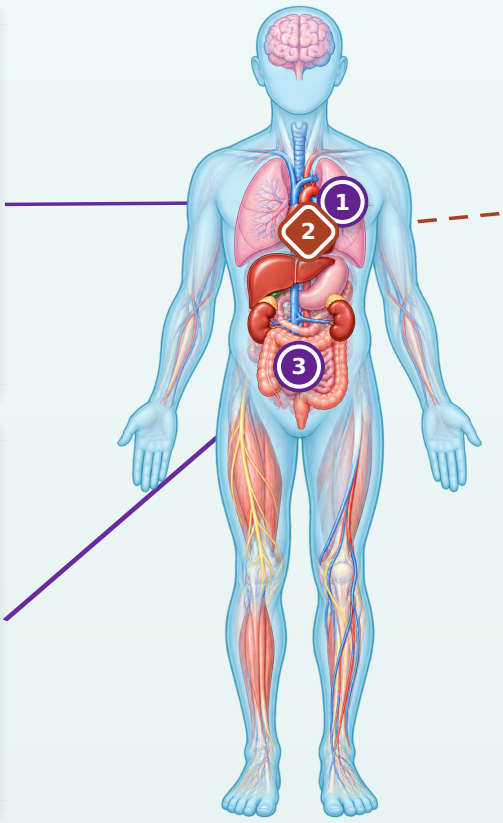
Trastuzumab

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Bowel / fatigue

Trastuzumab

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

Your record and contact card

Trastuzumab intravenous (IV) · every 3 weeks · National Cancer Control Programme (NCCP) 00200

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Trastuzumab IV · every 3 weeks · reusable treatment record · copy this sheet for another cycle of the same phase.

Trastuzumab · HER2-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Trastuzumab IV · every 3 weeks

NCCP 00200 v8 · 21-day cycle

IV trastuzumab every 21 days. Early breast cancer: complete one year total, allowing for earlier trastuzumab. Metastatic disease: continue while beneficial and tolerated; no automatic one-year stop.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00200/ · no patient information.

Draft for review · SACTCheck v0.84.14

SACTCHECK · CHAPTER 05

Sources and further information

Trastuzumab intravenous (IV) · every 3 weeks · National Cancer Control Programme (NCCP) 00200

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00200 v8 · official regimen ↗**
   https://healthservice.hse.ie/documents/6330/200_Trastuzumab_21days_Ubeecpc.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
7. **EMA Herceptin product information · cardiac assessment and formulation**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
8. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
9. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk